Patient Request for Corrections Project

Why did we choose <u>this</u> project as our first Patient Empowerment WG Focus?

The Volume and Impact of Errors is Staggering

Independent studies have found:

- Up to **95%** of medication lists had mistakes[1] [Geisinger 2014]
https://www.wsj.com/articles/health-care-providers-want-patients-to-read-medical-records-spot-errors-1402354902 **and** https://www.healthitoutcomes.com/doc/providers-encourage-shared-accountability-with-patients-0001#:~:text=%22If%20we%20don't%20have,and%20more%20effective%20medical%20care.

- **84%** of progress notes contained at least one documentation error[2] [AHIMA literature review, 2013] https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3797550/

- An average of **7.8 documentation errors per patient**[2]
[AHIMA literature review, 2013]

- Medical errors are **3rd leading cause of death** in US[3] [Hopkins 2016]
https://www.hopkinsmedicine.org/news/media/releases/study_suggests_medical_errors_now_third_leading_cause_of_death_in_the_us

2020 Open Notes Study [JAMA Network Open]

These clinicians KNEW patients could/would be reading their notes!

- **29,656** patients responded to survey
- **1 in 5** patients reported finding a mistake in their note
- **40%** perceived the mistake as **serious or very serious.**
  - Errors most commonly reported by patients as "very serious":
    - wrong or missing diagnoses
    - medical history
    - medications
    - physical examination
    - test results
    - wrong patient (!!)
    - wrong sidedness

JAMA Network Open | Original Investigation | Health Informatics

June 9, 2020

Frequency and Types of Patient-Reported Errors in Electronic Health Record Ambulatory Care Notes

Sigall K. Bell, MD[1,2]; Tom Delbanco, MD[1,2]; Joann G. Elmore, MD, MPH[3]; et al

Open Notes Study Conclusion: Invite patients to report errors!

Conclusions

The findings suggest that inviting patients to report perceived mistakes in shared visit notes, particularly those that patients believe are very serious, may be associated with improved record accuracy and patient engagement in diagnosis. Developing efficient mechanisms to respond to such reports appears to be important. At a time when patient demand for data is increasing along with federal support for providing patients easy access to health information, shared notes may help enlist patients, families, and practitioners in pursuing greater patient safety collaboratively.

Open Notes Study Conclusion: Need an efficient mechanism!

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Open Notes Study Conclusion: Fewer errors → greater safety

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Inequity: Errors have greater impact on minorities and those with poorer health

Minorities or those with poorer health are less likely to speak up to report errors [JAMIA 2021]

JAMIA
A SCHOLARLY JOURNAL OF INFORMATICS IN HEALTH AND BIOMEDICINE

Speaking up about patient-perceived serious visit note errors: Patient and family experiences and recommendations FREE

Barbara D Lam, Fabienne Bourgeois, Zhiyong J Dong, Sigall K Bell

"Participants who self-identified as Black or African American, Asian, "other," or "multiple" races, or those who reported poorer health were each less likely to speak up than White or healthier respondents.

The most common barriers to speaking up were not knowing how to report a mistake (61%) and avoiding perception as a "troublemaker" (34%)."

Interop means errors can propagate. Stop them at the source.

Interoperability mandates are improving the sharing of data between organizations.

Unfortunately, that means **errors get shared and propagate** at an increasing rate.

The Problem is Becoming More Visible

Government mandates are unlocking patients' data to allow them to fully view data in their chart.

Patients and advocates are now able to see the enormous amount of errors that could negatively impact their care.

There is now more need for a standardized process for patients to report errors than at any time in the past.

“Correction Principle” from ONC, 2008:

In its Nationwide Privacy and Security Framework for Electronic Exchange of Individually Identifiable Health Information, the ONC adopted the following principle on “Correction”:

> “Individuals should be provided with a timely means to dispute the accuracy or integrity of their individually identifiable health information (IIHI), and to have erroneous information corrected or to have a dispute documented if their requests are denied”

First released by ONC in December 2008 and has been subsequently included in the ONC Health IT Strategic Plan.

Recommendation Over a Decade Ago

Previous recommendations:

The Health IT Policy Committee (in 2011) was tasked to give recommendations that will help build public trust in health information technology and electronic HIE and enable their appropriate use to improve healthcare quality and efficiency.

Health IT Policy Committee recommended to ONC that they establish certification criteria to enable the HIPAA request for correction/amendment process.

https://www.healthit.gov/sites/default/files/facas/07_25_11_HITPC_Letter_PrivSecTigerTeam.pdf

Use of EHR Data in Research

With the advancement of FHIR, many research organizations (including Public Health) are increasing their use of EHR data.

Unless we provide an easy way for patients to make sure their data is correct, researchers will be using bad data.

Research using bad data can only result in bad research.

(Eroding public trust in health information technology and research)

Ability to Access is Incongruent with Current Ability to Correct

Access Electronic Health Information

- Standards based
- Modern technology – RESTful API
- Interactive – innovative personal health record functionality designed for ease of use.
- Scalable across multiple settings.

Request a Correction (current state)

- Non-standard
- Work intensive
- Low tech (fax! mail! phone!)
- Outside of the workflow
- Not interactive
- Does not scale

Patient Request for Corrections Project

Began: Summer 2020 **Ballot:** May 2022

4 Connectathons: January, May, Sept 2021, Jan 2022

- Received great feedback on draft IG testing different resources
- Grahame has given us clear direction for Sept 2021 connectathon
- Need more EHR involvement

Project Leads: Debi Willis and Virginia Lorenzi

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Input From: FHIR-I, Epic, Allscripts, and several other HL7 workgroups & members

What Can Be Done To Help Solve The Problem?

We need Involvement from the ONC to help move this forward:

- **Mandate the use of FHIR for patient request for corrections**.
  We NEED a standards-based processes to solve this problem.
- This will drive **more participation from EHR vendors to get this implemented**.

We need involvement from the public to help move this forward:

- Patients/advocates need to voice their concern about the overwhelming impact of errors in charts and the need for a standardized and easier way for patients to report errors.

Recap 1: The problem

- Real, widespread and serious – well documented in the literature.
- Impacts patient safety
- Known by the ONC
- Inequitable: more heavily impacts minorities and those with poorer health
- Impacts validity of research that uses EHR data
- Multiplying data is flowing faster, propagating errors

Recap 2: What Can Be Done

- **The ONC can establish certification criteria to enable the HIPAA request for correction/amendment process via FHIR**
  - FHIR resources exist that can be used to implement bi-directional communication. The IG is well under way.
  - There are people who are willing and able to work on this.
- **Patients and advocates need a way to express their concern and desire for this problem to be solved.**